

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 1

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	1 of 18–20 planned	Presenting Issue:	Contamination OCD + Checking behaviors

PRE-SESSION THERAPIST NOTES

New client, 26-year-old software engineer from Kerala, currently working in Bangalore. Self-referred after symptoms significantly worsened in the last 8–10 months. Reports strong contamination fears (especially touching surfaces, public places, shaking hands) leading to excessive hand washing (sometimes 30–40 times a day), avoiding touching door handles, and repeated checking of switches and locks. He feels exhausted, ashamed, and is increasingly avoiding social situations and office interactions. Risk appears low. Plan: Build rapport carefully, conduct a gentle assessment of obsessions and compulsions, provide psychoeducation about OCD as a treatable condition, introduce the CBT model for OCD, and set collaborative goals. Avoid starting ERP in Session 1.

SESSION START – BUILDING RAPPORT

Therapist: Good morning, Rohan. Thank you for coming in today. I know it can take a lot to reach out when things feel difficult. How are you feeling about being here?

Client: (speaking in a low, somewhat hesitant voice) I don't know... I've been thinking about this for months but kept putting it off. I feel a bit stupid coming here for this. It's just cleaning and checking... I should be able to control it myself.

Therapist: It's very common for people with OCD to feel that way — like they should just be able to stop. Many people feel ashamed or think it's "just a habit" they should overcome on their own. But what you're experiencing is actually a recognized and treatable condition. There's no need to feel stupid for being here. I'm glad you came.

Client: (nodding slightly) Thanks... I guess I just feel exhausted all the time. I keep thinking if I try harder, I can stop doing these things, but it only gets worse.

ASSESSMENT OF OBSESSIONS AND COMPULSIONS

Therapist: I'd like to understand what's been happening for you. You mentioned in your form that you have strong fears about contamination and do a lot of checking. Can you tell me a bit more about what that looks like on a typical day? You don't have to share anything you're not comfortable with.

Client: It's mostly about touching things. Like if I touch a door handle or shake someone's hand, I feel like my hands are dirty and I have to wash them immediately. Sometimes I wash them 10–15 times in one go until they feel "clean enough." At the office, I avoid touching anything if possible. I use tissue paper to open doors or press lift buttons. At home, I check the gas knob and switches multiple times before sleeping. If I don't, I keep thinking something bad will happen — like a fire or someone getting sick because of me.

Therapist: Thank you for sharing that so openly. It sounds incredibly exhausting and time-consuming. When you have the thought that something bad might happen if you don't check or wash, how strong does that feeling get, and what happens if you try to resist doing the compulsion?

Client: The anxiety becomes very high. My heart starts racing and I feel like I can't focus on anything else until I do the washing or checking. If I try to stop myself, the thought keeps coming back stronger, like "What if something actually happens because I didn't check?" So I end up doing it anyway. After I do it, I feel some relief, but it only lasts for a short time and then the thoughts come back.

Therapist: That cycle you just described — obsession (the scary thought), rising anxiety, doing the compulsion (washing/checking), and temporary relief — is very typical of OCD. The relief is short-lived, which is why the cycle repeats. This isn't about being "too clean" or "too careful." It's your brain sending false alarm signals that feel very real.

PSYCHOEDUCATION ABOUT OCD

Therapist: Would it be helpful if I explained a bit more about how OCD works? Many people find it useful to understand why these thoughts and behaviors feel so powerful even when, on some level, they know they might be excessive.

Client: Yeah... I keep thinking maybe I'm just overreacting or being paranoid. It would help to understand why this is happening.

Therapist: OCD involves two main parts: **obsessions** (unwanted, intrusive thoughts, images, or urges that cause anxiety) and **compulsions** (behaviors or mental acts you feel driven to perform to reduce that anxiety). In your case, the obsession is often something like "I might have touched something dirty and could make myself or others sick," and the compulsion is washing or checking. The problem is that doing the compulsion only reduces anxiety temporarily and actually strengthens the obsession over time because the brain learns that the compulsion "saved" you from danger. Does this match what you've been experiencing?

Client: Yes... exactly. Even though I know logically that washing my hands 15 times won't actually prevent anything bad from happening, in the moment it feels like I have no choice. And after I do it, I feel stupid for doing it, but then the next time the thought comes, it feels just as strong.

Therapist: That feeling of “I know it's irrational but I still have to do it” is extremely common in OCD. It's not a lack of logic or willpower. It's the brain's fear system being overactive and treating false alarms as real threats. The good news is that OCD is very treatable with the right approach — specifically a form of CBT called Exposure and Response Prevention (ERP). We don't have to start that today. Today is mostly about understanding what's happening and building a foundation.

COLLABORATIVE GOAL SETTING

Therapist: If therapy is helpful over the next few months, what would you like to be different in your daily life? Even small, realistic changes matter.

Client: I want to spend less time washing and checking. I want to be able to touch things without immediately feeling like I have to clean myself. I also want to stop avoiding social situations and office interactions because of this fear. And I want to feel less exhausted and ashamed all the time.

Therapist: Those are very clear and meaningful goals. We can work toward them step by step. In the beginning, we'll focus on understanding the OCD cycle better and building some coping strategies. Later, when you feel ready, we can gradually work on reducing the compulsions through ERP. You'll always be in control of the pace. How does that sound?

Client: It sounds okay. I'm still a bit scared about the ERP part, but I understand we won't start that immediately.

HOMEWORK ASSIGNMENT

Therapist: For this first week, I'd like to keep things simple and focused on understanding:

1. Keep a simple daily note of how many times you washed your hands or checked things more than “normal” (you don't have to write details if it feels difficult — just rough numbers and situations)
2. Notice what thoughts come just before you feel the urge to wash or check
3. Notice what happens to your anxiety level right after you complete the compulsion (does it go down? For how long?)

Client: I can try that. I already kind of know I wash a lot, but writing it down might help me see the pattern clearly.

Therapist: That's the idea. We'll use that information next session to understand your personal OCD cycle better. Any questions before we finish today?

Client: Not really... I feel a bit hopeful but also nervous about what comes next.

SESSION SUMMARY & FEEDBACK

Therapist: Today we talked about what's been happening with the contamination fears and checking, understood the OCD cycle, and set some initial goals. How are you feeling about today's session?

Client: It was okay. I feel like you understood what I was saying without judging me. That helped. I'm still nervous about treatment, but I want to try.

Therapist: That's completely fair. We'll go at your pace. I'll see you next week.

POST-SESSION THERAPIST NOTES

- Client presented with clear symptoms of Contamination OCD with checking behaviors. Significant shame and minimization were present initially but reduced as rapport built.
- Client demonstrated good insight into the irrationality of his fears on an intellectual level, but experiences strong anxiety-driven compulsions in the moment.
- Psychoeducation about the OCD cycle was well received. Client showed realistic nervousness about ERP but willingness to engage in treatment.
- Homework assigned: Monitoring of obsessions and compulsions (frequency, triggers, and anxiety levels).
- Next session: Review monitoring homework, deepen understanding of personal OCD cycle, continue building motivation and alliance, introduce more coping strategies before beginning ERP.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.